CONSULT FOR AFRICA
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PREPARED FOR

Clearview Fertility

For the attention of Dr Kunle Ajayi

GROWING CLEARVIEW FERTILITY / 11 OCTOBER 2026

Your laboratory is half empty. **The patients are not in Lekki.**

Nine years in, about a hundred people a month contact Clearview and fifteen start treatment. The laboratory runs at half what it could carry. The patients who would fill it are in mainland Lagos, in Abuja and Port Harcourt, in Accra, and in London and Houston, and they cannot get this done at home.

Consult for Africa and Mezo would go and get them, and be paid only when they arrive.

Indicative throughout. The commercial terms are fixed once we have your numbers, and not before. Nothing here is for signing.

OUR FEE

N950,000

per patient who starts treatment. One fifth of a single cycle, paid once. Nothing on her second cycle, her storage or her delivery.

No patient, no fee.

Indicative. Fixed at stage two.

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What another year of this costs you

Every figure below comes from the assumptions in your strategy note and will move once you send us the real ones. The shape will not move.

	What is happening	What it is worth
1	About 120 people contact you in a month and about 15 start treatment. The other 105 go somewhere else, or nowhere.	Five points of conversion is N342 m a year
2	Your laboratory can run about 30 cycles a month. It runs about 15.	Half of the most expensive asset you own, idle
3	You create around 62 pregnancies and 37 caesarean sections a year.	N139 m a year of obstetric work, and we do not know whose hospital bills it
4	Embryos sit in your tank and nobody telephones the patient.	N11.4 m a month at the best margin in the building

None of that is a marketing problem and none of it needs a new patient. It is demand you already generate and already pay for, leaking out of a business that has worked for nine years on your name alone.

You are fishing in the most crowded water in West Africa

Lekki is an excellent destination and a poor catchment. Nordica, The Bridge Clinic and Medical ART Center have twenty years and national recognition, and every new boutique on this axis is fighting for the same six kilometres. The people who cannot get this done at home are not.

Where they would come from	What stops them today	Four cycles a month is
Mainland Lagos	Traffic, and fourteen monitoring visits	N228 m a year
The rest of Nigeria	Relocating to Lagos for three weeks	N262 m a year
West Africa	Language, payment, and no reason to believe us yet	N285 m a year
The diaspora	Trust, entirely	N365 m a year

> **A woman in Abuja or Accra or Houston is not choosing between you and Nordica. She is choosing between you and nothing.**

The one thing none of them can copy

- **She cannot live in Lagos for three weeks.** A stimulation cycle needs near daily scans. Monitoring done near her home turns a three week relocation into a few days here. The Mezo network already has scan sites on the mainland, Abuja is next, and almost nobody in Nigeria runs this properly.
- **She is frightened of being unwell far from home.** Ovarian hyperstimulation declares itself several days after retrieval, which is exactly when she has flown home to a doctor who does not know what she was given. That single fear is why fertility travel in this region stays small.
- **You own a hospital.** So you can tell her that if anything goes wrong she is admitted by your own team and it is already in the price. No fertility clinic in Lekki can say that sentence.
- **And they cannot buy their way to it.** A clinic would have to buy a hospital and recruit obstetricians. A hospital would have to build an embryology laboratory and find an embryologist, which takes years. You are an obstetrician who already owns both.

A fertility patient is not one cycle, and the rest of her is yours

What she is worth to Clearview	NGN m	Who is paid on it
Her first fresh cycle	4.75	We take one fifth of this. Once.
A second cycle, 40% of the time	1.90	Yours. We are paid nothing on it.
A frozen transfer, 30% of the time	0.48	Yours. We are paid nothing on it.
Two years of embryo storage	0.18	Yours. We are paid nothing on it.
Her delivery at Clearview Hospital, 30% of the time	0.67	Yours. We are paid nothing on it.
What one patient is worth over 24 months	7.99	

We are paid on the hardest part, which is getting her through the door the first time. Everything that follows, and it is more than half of what she is worth, is yours without a further naira to us. On her whole journey our fee is about 26 per cent of your own contribution.

One packed day, and we take everything we can get

Everybody comes to our office on the same day and we film until we run out of daylight. The point is volume: the more testimonials and the more scripted answers we get in the room, the longer the library lasts and the fewer days of your life this costs.

What we take	Per day	Why
Patient testimonials	8 to 12	Consented, filmed properly, the whole point of the day
Scripted pieces from you	20 to 30	Short answers to the questions patients actually ask
Your team and referring colleagues	4 to 6	Embryologist, nurses, a gynaecologist who refers
Edited assets from the above	60 to 80	Cutdowns, vertical versions, captioned variants

Two of these days a year is the whole of your time on camera. Everything you see running for the next twelve months, in the adverts and in the blog pieces, comes out of those two days.

The one thing we cannot manufacture is you

Fertility is bought on trust, from a named doctor, by a woman who has already been disappointed. No agency can invent that. It has to come out of your head and your clinic, and then we do the rest.

- **Two days of filming a year.** At our office, which is already set up for it. We ask the questions, we leave, and that is the whole of your time on camera. You never edit anything and you never post anything.
- **Patients willing to tell their story.** Recorded in the same session, properly consented in writing, and never used beyond what they agreed to. One real voice does more than a year of stock photography.
- **Your team, and the colleagues who refer to you.** Your embryologist, your nurses, a gynaecologist who sends you patients. It is the same shoot day and it costs nothing extra.
- **The questions your patients actually ask.** From you and from your front desk. The ones asked at three in the morning are the ones worth answering publicly.
- **Your honest position on the hard things.** Age and odds. When IVF is not the right next step. What a cycle really costs. This is the content nobody in this market makes, and it is the content that converts.
- **A decision inside a week.** On anything we send you for approval. Campaigns die in approval queues.

Adverts buy attention. Blogs buy belief.

- **She will search you before she calls.** Nobody spends four million naira on a clinic they found in an advert without going looking first. What she finds at that moment decides whether she picks up the phone.
- **A placed piece outlives the campaign.** An advert stops the day you stop paying for it. An article on a Nigerian health or lifestyle blog keeps ranking in search for years, and it keeps working in the months when nothing is running.
- **It is written out of the same shoot day.** The scripted answers you give on camera become the articles. You are not asked for anything additional, and we are not charging you a second time to produce them.
- **5 placements a campaign, at cost.** Typically N0.15 m to N0.80 m each depending on the title. We place them, we write them, and we take no commission on what you pay the publisher.

Adverts find her when she is ready. Search finds her a year earlier.

Clearview is already a partner on Mezo, our patient platform. Alongside the paid campaigns, Mezo would build the search side around you, at its own cost, and be paid the same single fee if a patient it brings starts treatment.

- **She searches the worry before she searches the clinic.** "Can't get pregnant after a year", "HSG price Lagos", "low sperm count". Nobody in Lagos answers those properly, and the couple asking them has not chosen a clinic yet.
- **The Couple Fertility Check is the front door.** Both partners, one morning, one fixed price: consultation, scan, AMH, HSG and semen analysis. The result points to one next step, and if that step is IVF the fee counts towards the cycle.
- **Your price, published as yours, drugs included.** Every competitor publishes a cycle price with the drugs left out. Yours would be the only all-in figure in the market, at exactly what you charge. Mezo adds nothing to it.
- **A page for the patient abroad.** Days in Lagos, the price in pounds and dollars, monitoring near home, and the hospital behind the laboratory. Written for London and Houston separately, because their reasons are different.
- **Paid on the platform, so attribution is settled.** She books and pays through Mezo, so where she came from is on the receipt. There is nothing to argue about.

No patient from this channel is counted anywhere else in this document. Search takes three to six months to earn, and we will not model what we have not measured.

YEAR ONE

Two shoot days. Four campaigns. Twelve months live.

What you spend	NGN m
Content: 2 shoot days at N3.31 m	6.62
Paid advertising: N3.0 m a month across 12 live months	36.0
PR: 20 placements on Nigerian blogs	5.0
Your whole outlay for the year	47.6

What comes back	
Qualified enquiries it produces	1,640
Patients who go on to treatment	115
What they pay Clearview over 24 months each, NGN m	922
Our fee: 115 patients at N950,000, NGN m	110
Clearview keeps, NGN m	765

You commit the N47.6 m at the top and nothing else. The fee below it falls due only once patients have paid you.

Judge this on the bad case, not the middle one

Year one	Patients	They pay you	Our fee	You keep
If it goes badly	46	N311 m	N44 m	N220 m
If it goes as modelled	115	N922 m	N110 m	N765 m
If it goes well	180	N1,613 m	N171 m	N1,395 m
If nobody comes	0	N0	N0	(N47.6 m)

Even in the bad case, a year of content, advertising and PR costing N47.6 m returns N311 m over two years. The worst outcome on the table is the last row, and it costs you N47.6 m.

Every figure here is an assumption until the first campaign replaces it with your own cost per patient. That is the honest position, and it is why we are willing to be paid on the outcome rather than asking you to buy the forecast.

Five lines. You pay for three, and we carry two.

	What it is	Who pays
Content	Two shoot days a year, each a full packed day at the C4A office with everybody in: you, consented patients, your team, the colleagues who refer to you. N3.31 m a day, so N6.62 m for the year, and that is the only fixed fee here.	You, to us
Running it	Building, running, optimising and refreshing every campaign off that library. We absorb this. We are paid on conversion, so keeping it working is how we get paid.	Nobody. It is ours.
Media and PR	Paid advertising plus placed coverage on Nigerian blogs. At N3.0 m a month of ads and 5 placements a campaign, that is N41.0 m across the year. Passed through at cost, and we take no commission on it.	You, to the platforms
Search and Mezo	The organic side: search pages for the whole journey, the Couple Fertility Check, the diaspora page and the Mezo network for remote scans. Mezo builds and runs it at its own cost. No media, no content fee.	Nobody. It is ours.
Our fee	N0.95 m for each attributed patient who starts treatment, from either channel, paid once, when she pays you. One fifth of a single cycle. Everything she spends after that first cycle is entirely yours. If nobody comes, we are paid nothing.	You, to us, only when it works

Every agency in Lagos takes fifteen to twenty per cent of your media budget, which pays them more the more you spend, whether or not a single patient arrives. We take nothing on media and carry our upside on the only number that matters to you.

Two days a year, priced openly

One packed shoot day	Days	Rate	NGN m
Crew for a full packed day: camera, lighting, sound, assistant	1.2	0.450	0.540
Studio set and strike at the C4A office	1	0.120	0.120
Creative direction, scripting and scheduling the contributors	3	0.150	0.450
Consent and compliance across every patient contributor	1.5	0.120	0.180
Post production: one packed day into a deep asset library	14	0.085	1.190
Archiving, tagging and library management	2	0.085	0.170
Direct cost			2.650
Our margin, 25 per cent			0.662
Per shoot day			3.31
Two days, the whole year			6.62

Post production is the largest line because the day is packed, and that is the right trade: crew time is expensive and editing is not. We shoot at our own office, so you are not paying for a studio.

Each library feeds two three month campaigns, so two days covers twelve months of advertising. Creative does tire eventually, and when it does we reshoot at our own cost, because we are paid when it converts and not before.

One question a month, and you can audit every answer

Because the fee is a flat amount when she starts treatment, there is only ever one thing to agree: did this patient come from a campaign we ran. No share of your revenue, no look at your books, and no argument two years later about who her second cycle belonged to.

	The rule
What counts	A patient whose first contact with Clearview came through a campaign we ran, and who then starts and pays for treatment.
What never counts	Anyone already in your records, anyone a colleague referred, and anyone who walks in. Your existing business is yours and we do not touch it.
How it is tagged	Every enquiry logged at source, by the register we build in week one, plus the question asked at booking. A patient who books through Mezo carries her source on the payment itself. If the source is unclear, it does not count.
When we invoice	When she has paid you, and not before.
Anything arguable	Goes your way. We would rather lose a fee than have this conversation every month.

We will not quote you a number we cannot yet count

Any agency will put a price in front of you this week. We will not, and the reason is not caution. It is that we are the ones carrying the risk in this arrangement, so we are the ones who need the numbers. Ten of them, and you already have all ten.

	What we need	What it decides
1	Fresh cycles in a normal month, and in a bad one	The baseline everything is measured against
2	All in price of a cycle, and whether drugs are inside it	What one patient is worth, which sets our fee
3	Enquiries a month, and how many ever cycle	Whether this is a demand problem or a conversion problem
4	Live birth rate by age band, from your records	What may honestly be claimed, and the repeat rate
5	Share who return for a second cycle or a frozen transfer	Most of the patient value sits here
6	Patients with embryos stored, and whether storage is billed	A revenue line you may already own
7	Share of your IVF pregnancies that deliver with you	The largest single number in the model
8	Retrievals the laboratory could run in a week	The ceiling on the whole plan, and on the media budget
9	Where your patients travel from today	Whether inbound is new or already happening quietly
10	What you spend on marketing now, and what it produces	The only real read we have on your cost per enquiry

Three stages, and nobody is committed until the third

	What happens	What you commit	What we commit
One	You send the ten numbers. The five surveys run, and they are already live. Two to three weeks.	Nothing. No fee and no contract.	Nothing charged. We read it and we tell you honestly what we think.
Two	We price it properly off your numbers rather than our assumptions, and put the final terms in front of you.	Nothing. You may walk away here having paid nothing and still keep the analysis.	A firm price, held open for thirty days.
Three	First shoot day, then the campaign goes live.	The shoot day and the media. That is your whole exposure.	Everything else, at our own risk.

What we can price today is what we control: a shoot day is N3.31 m because that is what a shoot day costs, and we take no commission on media whatever your numbers say. What we will not fix today is the part that depends on how your business actually behaves.

Every figure elsewhere in this document is indicative and built on assumed volumes. It is here to show you the shape of the arrangement, not to be signed.

NEXT

This week, and it costs you nothing

	What	Who
1	Push the five surveys out. They are live at consultforafrica.com/ClearviewProject. The one for people who enquired and did not proceed is the most useful and needs the most time.	Your team
2	Send the ten numbers on page 15. Estimates are fine. Where you do not track something, say so, because that is itself an answer and usually the most useful one.	You
3	Approve how Clearview appears on Mezo: your name, your cycle prices as you set them, and the consultant who reviews the pages. The search work starts on that alone, because it takes months to rank and costs you nothing.	You
4	We come back with the real model, priced off your business, and you decide.	Us

There is nothing to sign and nothing to pay at this stage. If the numbers come back and the case is not there, I will tell you so, and that will have cost you a fortnight and no money at all.

CONSULT FOR AFRICA
Build. Transform. Grow.

We will not charge you for spending your money.
We will charge you for filling your theatre.

You pay for the production and the media. We are paid when a patient our work brought in pays you. If they do not come, that line is zero.

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